

# Well Within

## Clinical Rules, Pattern Boundaries, and Fixture Review

|                |                                                                             |
|----------------|-----------------------------------------------------------------------------|
| Prepared for   | A qualified current Creighton/FABM clinical reviewer                        |
| Review purpose | Adjudicate exact software rules, supported scope, and permitted terminology |
| Implementation | Well Within rules engine at commit 261fc0f                                  |
| Packet version | 1.0 - July 12, 2026                                                         |

**What we are asking** Please review the stated current behavior against authorized sources and your clinical judgment. For each decision, approve the behavior for a clearly defined scope, specify a change, require a review/support state, or hold it pending evidence.

**What we are not asking** This is not a request for blanket endorsement, certification, affiliation, diagnosis, ovulation confirmation, pregnancy prediction, or avoiding-pregnancy guidance. Legal, regulatory, privacy, trademark, copyright, and licensing decisions are routed separately to counsel.

Confidentiality note: This packet contains product behavior and synthetic review scenarios only. It includes no private user charts or personal health information.

# 1. Reviewer orientation

Well Within is a local-first mobile charting app. Users record daily bleeding and mucus observations. A deterministic, network-free rules engine organizes those observations into chart markers and retrospective summaries. The app does not claim to diagnose a condition, confirm ovulation, predict pregnancy, or replace individualized instruction.

## The narrow review objective

- Determine which exact observation mappings and transformations are clinically appropriate for an unassisted app.
- Define the population and chart patterns for which any automated retrospective marker is supportable.
- Decide when the app should continue showing observations without an automated conclusion.
- Approve, revise, or prohibit the ordinary-user terms listed in this packet.
- Identify the authorized source edition and page for every method-specific rule that is approved.

## Requested review method

1. Review each priority decision independently; do not infer approval from the current implementation.
2. Mark one response category and write any required change, limitation, or source citation.
3. Review the fixture set and annotate the expected gold output for each applicable scenario.
4. Provide at least 20 de-identified, authorized gold charts spanning supported and unsupported cases before final production approval.
5. Join one adjudication session with product and engineering to resolve conflicts and version the approved decision record.

## Response categories

**Approve** — Appropriate for the stated population and pattern scope.

**Approve with changes** — Use only after the reviewer's written change is made.

**Review/support state** — Keep observations visible; do not choose a marker automatically.

**Hold** — No automated output until authorized evidence is supplied.

**Evidence standard** Repository specifications and automated tests show implementation consistency. They do not substitute for authorized method sources, qualified clinical judgment, or product-specific validation.

## 2. Current implementation and interim safeguards

The following describes what the current product does. It is not presented as a clinically approved model.

### Processing sequence

- **Observation:** the user records bleeding, sensation, appearance, frequency, notes, or an intentionally not-observed day.
- **Normalization:** the engine derives a daily mucus classification, internal rank, bleeding class, and optional generated notation.
- **Marker logic:** the engine evaluates a possible Peak-type day and a three-lower-calendar-days sequence.
- **Capability state:** the app decides whether a retrospective summary is available, still forming, limited by missing data, or not selected because more than one sequence qualifies.
- **Presentation:** Calendar, Cycle History, Help, and PDF exports consume the same support state and suppress ineligible derived conclusions.

### Current capability states

| State              | Current trigger                                                                  | Current user path                                                                    |
|--------------------|----------------------------------------------------------------------------------|--------------------------------------------------------------------------------------|
| forming            | The record does not yet support a retrospective summary.                         | Names the observations or developing pattern and asks the user to keep charting.     |
| summary available  | One sequence satisfies the current software rule.                                | Names the marked Peak Day, the three following days, and the chart-only limitation.  |
| blocked by missing | A gap or not-observed day affects the current boundary or confirmation sequence. | Explains the specific limitation; charting and editing remain available.             |
| review recommended | More than one separated sequence independently satisfies the current rule.       | Does not select one Peak Day; outside support is optional and never unlocks the app. |

### Engineering changes already completed

- Displayed cycle days, Peak days, durations, comparisons, overlays, and PDF rows now use elapsed calendar dates rather than stored-row indexes.
- The unsupported historical shortcut that estimated fertile opening as Peak minus five days was removed.

- High/Moderate/Low confidence language was replaced with capability and completeness language.
- Missing/review states suppress derived summaries across Cycle Detail, History, comparisons, overlays, and default PDF output while preserving observations and export access.
- Charting, editing, history, and observation export remain available in every support state.

**Verification context** At packet creation, 184 core rules tests and 53 mobile tests pass; CI coverage is 97% statements, 92.33% branches, 100% functions, and 98.66% lines. This proves internal consistency only, not clinical validity.

### 3. Priority clinical decision records

Complete every record. If approval depends on a different population, pattern, training relationship, or source edition, write that condition explicitly.

#### Decision map

| ID   | Decision                                                       |
|------|----------------------------------------------------------------|
| D-01 | Daily observation reduction                                    |
| D-02 | Sensation, appearance, and generated notation                  |
| D-03 | Contradictory or incomplete input combinations                 |
| D-04 | Flow, spotting, brown observations, and mucus                  |
| D-05 | Cycle start and continuing flow                                |
| D-06 | Peak-type observation definition                               |
| D-07 | Retrospective Peak Day and the three-day sequence              |
| D-08 | Calendar gaps and days marked not observed                     |
| D-09 | Separated or repeated Peak-type sequences                      |
| D-10 | Continuous mucus, non-Peak-only patterns, and special contexts |
| D-11 | Permitted ordinary-user terminology                            |
| D-12 | History comparisons and inclusion eligibility                  |

## D-01 Daily observation reduction

|                  |                                                                                                                                                                                         |
|------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Audit links      | R-01; I-08                                                                                                                                                                              |
| Why it matters   | When a user records more than one observation on the same day, the app must decide what is retained and what can drive a derived marker.                                                |
| Current behavior | The engine normalizes each observation, then uses the highest internal 0-3 rank as the day's interpretive value. The original observations remain stored.                               |
| Evidence state   | Public materials support recording the strongest sign in broad terms. The app's rank abstraction, tie handling, and invalid-combination behavior do not yet have source-level approval. |

### Questions for the reviewer

- Is strongest-sign reduction appropriate for every supported user and cycle context?
- When two observations conflict, should the app select, preserve both without interpretation, or ask the user to correct the entry?
- Which original details must remain visible to a reviewer even when one value drives the automated summary?

### Reviewer decision

☐ Approve as implemented for the stated scope

☐ Approve with the changes written below

☐ Do not interpret automatically; use a defined review/support state

☐ Hold pending an authorized source or additional evidence

Required change / limitation: \_\_\_\_\_

Authorized source, edition, and page: \_\_\_\_\_

Reviewer initials and date: \_\_\_\_\_

## D-02 Sensation, appearance, and generated notation

|                  |                                                                                                                                                                                                                                        |
|------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Audit links      | R-02-R-05; R-07; I-01-I-03; I-09                                                                                                                                                                                                       |
| Why it matters   | The mapping from recorded words to generated notation and internal ranks determines every later summary.                                                                                                                               |
| Current behavior | The app maps sensations, appearances, and frequency into generated notation and a 0-3 interpretive rank. Damp, wet, and shiny inputs can currently open the derived mucus pattern.                                                     |
| Evidence state   | The repository contains an unsourced internal table and a direct contradiction about whether several lower-numbered observations represent mucus or non-mucus sensations. Authorized source edition and page-level review are missing. |

### Questions for the reviewer

- Using an authorized source, which mappings are clinically correct for software entry and export?
- Should damp, wet, or shiny without a mucus appearance open any automated pattern boundary?
- Does omitted frequency affect interpretation, export notation only, or entry validity?

### Reviewer decision

☐ Approve as implemented for the stated scope

☐ Approve with the changes written below

☐ Do not interpret automatically; use a defined review/support state

☐ Hold pending an authorized source or additional evidence

Required change / limitation: \_\_\_\_\_

Authorized source, edition, and page: \_\_\_\_\_

Reviewer initials and date: \_\_\_\_\_

## D-03 Contradictory or incomplete input combinations

|                  |                                                                                                                                                                                                        |
|------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Audit links      | R-06; I-04-I-06; I-10                                                                                                                                                                                  |
| Why it matters   | A polished interface can still accept combinations that a trained practitioner would regard as internally inconsistent.                                                                                |
| Current behavior | Some appearance selections can override a 'dry' sensation and become Peak-type in the current engine. Missing dates are tolerated in legacy/test paths, while production records normally carry dates. |
| Evidence state   | No documented clinical decision defines which combinations are valid, which require correction, or which may be recorded but excluded from interpretation.                                             |

### Questions for the reviewer

- For dry plus clear, lubricative, or yellow appearance, should the app accept, warn, require correction, or record without interpretation?
- Which fields are mandatory before a day may participate in automated interpretation?
- What recovery message is appropriate for invalid, duplicate, missing, or out-of-order dates?

### Reviewer decision

☐ Approve as implemented for the stated scope

☐ Approve with the changes written below

☐ Do not interpret automatically; use a defined review/support state

☐ Hold pending an authorized source or additional evidence

Required change / limitation: \_\_\_\_\_

Authorized source, edition, and page: \_\_\_\_\_

Reviewer initials and date: \_\_\_\_\_

## D-04 Flow, spotting, brown observations, and mucus

|                  |                                                                                                                                                                                                         |
|------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Audit links      | R-09-R-12; F-06; I-07; C-10-C-11                                                                                                                                                                        |
| Why it matters   | Users can record mucus during bleeding, but the current interpretation treatment differs by bleeding class.                                                                                             |
| Current behavior | Menstrual flow suppresses Peak-type interpretation on that day. Spotting remains descriptive. Light/spotting rows are currently blocked from opening the derived interval or becoming a Peak candidate. |
| Evidence state   | Public method material indicates mucus is observed on light/very-light flow days, but the correct automated treatment and wording have not been approved.                                               |

### Questions for the reviewer

- Can a light-flow or spotting day influence a Peak-type marker, and under what exact conditions?
- Which bleeding observations are descriptive only, and which require individualized review?
- What should the ordinary user see when bleeding and a high-quality mucus sign occur together?

### Reviewer decision

☐ Approve as implemented for the stated scope

☐ Approve with the changes written below

☐ Do not interpret automatically; use a defined review/support state

☐ Hold pending an authorized source or additional evidence

Required change / limitation: \_\_\_\_\_

Authorized source, edition, and page: \_\_\_\_\_

Reviewer initials and date: \_\_\_\_\_



## D-05 Cycle start and continuing flow

|                  |                                                                                                                                                                                                                                   |
|------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Audit links      | R-21-R-22; F-07; C-12-C-14                                                                                                                                                                                                        |
| Why it matters   | Every cycle day, Peak day, duration, comparison, and export depends on the cycle-start boundary.                                                                                                                                  |
| Current behavior | The current engine begins a cycle at a heavy/moderate flow row that is not treated as continuing prior flow. Calendar-day math is now used for displayed day numbers and durations, but boundary adjudication remains unresolved. |
| Evidence state   | Public descriptions refer broadly to onset of menstruation; the exact heavy/moderate threshold, leading light days, missing dates within flow, and charts without heavy/moderate flow lack qualified approval.                    |

### Questions for the reviewer

- For light -> heavy -> moderate, which date is Cycle Day 1 and which dates belong to the cycle?
- How should a missing calendar date between heavy/moderate observations affect continuing-flow logic?
- What should the app do when no heavy/moderate boundary exists?

### Reviewer decision

☐ Approve as implemented for the stated scope

☐ Approve with the changes written below

☐ Do not interpret automatically; use a defined review/support state

☐ Hold pending an authorized source or additional evidence

Required change / limitation: \_\_\_\_\_

Authorized source, edition, and page: \_\_\_\_\_

Reviewer initials and date: \_\_\_\_\_

## D-06 Peak-type observation definition

|                  |                                                                                                                                                                                           |
|------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Audit links      | R-13; C-13; I-01-I-08                                                                                                                                                                     |
| Why it matters   | The app must separate what the user recorded from the later retrospective Peak Day marker.                                                                                                |
| Current behavior | Clear, cloudy/clear, lubricative, stretchy, and some promoted combinations currently produce the highest internal rank and may be labeled Peak-type based on what was recorded.           |
| Evidence state   | Clear, stretchy, and lubricative signs are broadly supported in public and peer-reviewed descriptions. Exact combination handling and the cloudy/clear mapping require authorized review. |

### Questions for the reviewer

- Which exact observations may be called 'Peak-type' in an unassisted app?
- Is 'Peak-type observation based on what you recorded' acceptable ordinary-user wording?
- Which observations must never be promoted to Peak-type automatically?

### Reviewer decision

☐ Approve as implemented for the stated scope

☐ Approve with the changes written below

☐ Do not interpret automatically; use a defined review/support state

☐ Hold pending an authorized source or additional evidence

Required change / limitation: \_\_\_\_\_

Authorized source, edition, and page: \_\_\_\_\_

Reviewer initials and date: \_\_\_\_\_

## D-07 Retrospective Peak Day and the three-day sequence

|                  |                                                                                                                                                                                                                                                                      |
|------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Audit links      | R-14-R-16; C-01; C-05; C-15                                                                                                                                                                                                                                          |
| Why it matters   | This is the central derived marker currently surfaced throughout Calendar and Cycle History.                                                                                                                                                                         |
| Current behavior | A non-flow highest-rank day becomes a candidate. The app marks it as Peak Day only after the next three consecutive calendar days are present, observed, and each lower than the candidate. A later highest-rank day during the waiting period resets the candidate. |
| Evidence state   | The last clear/stretchy/lubricative day and P+3 framework are publicly described. The strict software comparison, reset behavior, and permitted patient-facing language have not been clinically approved for this implementation.                                   |

### Questions for the reviewer

- Is the strict 'each of the next three days is lower than the candidate' rule correct for the proposed scope?
- Does a gradual decline require any relationship among P+1, P+2, and P+3 beyond all being lower than the candidate?
- Which terms are acceptable: candidate, identified Peak Day, marked Peak Day, P+1/P+2/P+3, post-Peak pattern?

### Reviewer decision

☐ Approve as implemented for the stated scope

☐ Approve with the changes written below

☐ Do not interpret automatically; use a defined review/support state

☐ Hold pending an authorized source or additional evidence

Required change / limitation: \_\_\_\_\_

Authorized source, edition, and page: \_\_\_\_\_

Reviewer initials and date: \_\_\_\_\_

## D-08 Calendar gaps and days marked not observed

|                  |                                                                                                                                                                                                                                                                                                          |
|------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Audit links      | R-25; C-02-C-04                                                                                                                                                                                                                                                                                          |
| Why it matters   | The app should be conservative without trapping users or misrepresenting an intentionally unobserved day as an app error.                                                                                                                                                                                |
| Current behavior | A missing calendar date or explicit not-observed day inside the three-day confirmation sequence blocks the Peak summary. An earlier gap can limit the pattern boundary. Future days beyond the last record are treated as a developing pattern, not a gap. Charting and editing always remain available. |
| Evidence state   | This behavior is internally verified and deliberately conservative; exact clinical effect and recovery wording still need review.                                                                                                                                                                        |

### Questions for the reviewer

- Should an absent date and an explicit not-observed row have the same clinical effect?
- Which gaps limit only the opening boundary, which block Peak, and which make the full pattern unsupported?
- Is the proposed guidance appropriate: keep charting, restore a remembered open day, and preserve an intentionally not-observed day?

### Reviewer decision

☐ Approve as implemented for the stated scope

☐ Approve with the changes written below

☐ Do not interpret automatically; use a defined review/support state

☐ Hold pending an authorized source or additional evidence

Required change / limitation: \_\_\_\_\_

Authorized source, edition, and page: \_\_\_\_\_

Reviewer initials and date: \_\_\_\_\_

## D-09 Separated or repeated Peak-type sequences

|                  |                                                                                                                                                                                                                                                                   |
|------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Audit links      | R-17; F-05; C-09                                                                                                                                                                                                                                                  |
| Why it matters   | Selecting the first or last sequence without an approved rule could create a clinically consequential false conclusion.                                                                                                                                           |
| Current behavior | When more than one separated sequence independently satisfies the current three-lower-days rule, Well Within shows 'Your chart shows more than one possible Peak pattern' and does not choose one Peak Day. Outside review is optional and never unlocks the app. |
| Evidence state   | Repeated/double-Peak behavior is outside the implemented method scope. The current review state is a conservative product decision, not a clinical classification.                                                                                                |

### Questions for the reviewer

- Is non-selection plus optional review the correct default for two independently qualifying sequences?
- Are there any patterns where an automated app may select the later sequence? State the exact prerequisites.
- What wording avoids diagnosis while accurately explaining why one Peak Day is not shown?

### Reviewer decision

☐ Approve as implemented for the stated scope

☐ Approve with the changes written below

☐ Do not interpret automatically; use a defined review/support state

☐ Hold pending an authorized source or additional evidence

Required change / limitation: \_\_\_\_\_

Authorized source, edition, and page: \_\_\_\_\_

Reviewer initials and date: \_\_\_\_\_

## D-10 Continuous mucus, non-Peak-only patterns, and special contexts

|                  |                                                                                                                                                                                                                                                              |
|------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Audit links      | R-19-R-20; C-06-C-08; special-context set                                                                                                                                                                                                                    |
| Why it matters   | A standard dry-to-mucus-to-Peak model must not silently imply that every chart follows that pattern.                                                                                                                                                         |
| Current behavior | Continuous Peak-type signs remain a developing pattern. Continuous lower-quality mucus and non-Peak-only runs do not receive a confirmed Peak summary. No BIP, postpartum, perimenopause, medication, infection, or diagnostic mechanics have been invented. |
| Evidence state   | Public materials describe BIP and special-context complexity, but this app has no approved interpretation path or population scope for those patterns.                                                                                                       |

### Questions for the reviewer

- For each special context, choose: full interpretation, restricted interpretation, defined caution, or no interpretation/referral.
- What minimum history or practitioner-established BIP is required before any automated summary?
- Which neutral message should appear when the app intentionally does not interpret the pattern?

### Reviewer decision

☐ Approve as implemented for the stated scope

☐ Approve with the changes written below

☐ Do not interpret automatically; use a defined review/support state

☐ Hold pending an authorized source or additional evidence

Required change / limitation: \_\_\_\_\_

Authorized source, edition, and page: \_\_\_\_\_

Reviewer initials and date: \_\_\_\_\_

## D-11 Permitted ordinary-user terminology

|                  |                                                                                                                                                                                                                                                                                                                        |
|------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Audit links      | C-01-C-19; R-18; R-31-R-32                                                                                                                                                                                                                                                                                             |
| Why it matters   | Users need understandable language without converting a chart marker into ovulation, biological certainty, or contraceptive guidance.                                                                                                                                                                                  |
| Current behavior | The interim UI uses observational language: possible Peak Day, chart marks a Peak Day after three lower days, post-Peak pattern, missing-data limitation, or multiple possible Peak patterns. Confidence tiers, 'most fertile day confirmed,' and broad current fertile-window conclusions were removed or suppressed. |
| Evidence state   | The wording is product-reviewed and internally consistent, but it is not clinically or legally approved. 'Fertile Start,' 'Fertile End,' 'total fertile days,' and equivalent claims remain held.                                                                                                                      |

### Questions for the reviewer

- Approve, revise, or prohibit each proposed term in the terminology worksheet.
- Specify the exact limitation that must appear next to any retrospective Peak Day statement.
- Identify any wording that could improperly support avoiding-pregnancy or diagnosis decisions.

### Reviewer decision

☐ Approve as implemented for the stated scope

☐ Approve with the changes written below

☐ Do not interpret automatically; use a defined review/support state

☐ Hold pending an authorized source or additional evidence

Required change / limitation: \_\_\_\_\_

Authorized source, edition, and page: \_\_\_\_\_

Reviewer initials and date: \_\_\_\_\_

## D-12 History comparisons and inclusion eligibility

|                  |                                                                                                                                                                                                                                                                                              |
|------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Audit links      | R-29-R-30; C-21-C-22                                                                                                                                                                                                                                                                         |
| Why it matters   | Retrospective comparisons can feel predictive or normative if thresholds, sample size, and eligible-cycle rules are unclear.                                                                                                                                                                 |
| Current behavior | Only cycles with the summary-available support state contribute to derived aggregates, comparison context, and Peak overlays. Active-cycle anticipatory timing is suppressed. Calendar-correct dates are used. Existing comparison bands remain product thresholds without clinical meaning. |
| Evidence state   | No clinical source approves the current +/- day thresholds or terms such as usual, consistent, or significant variation.                                                                                                                                                                     |

### Questions for the reviewer

- Which comparisons are purely descriptive and appropriate for ordinary users?
- What minimum eligible sample size and exclusions are required?
- Should comparison bands be removed, numerically displayed without labels, or clinically approved against evidence?

### Reviewer decision

☐ Approve as implemented for the stated scope

☐ Approve with the changes written below

☐ Do not interpret automatically; use a defined review/support state

☐ Hold pending an authorized source or additional evidence

Required change / limitation: \_\_\_\_\_

Authorized source, edition, and page: \_\_\_\_\_

Reviewer initials and date: \_\_\_\_\_



## 4. Clinical fixture worksheet

For every applicable fixture, independently mark the valid daily inputs, cycle start, cycle-day numbers, relevant mucus boundary, Peak-type observations, Peak candidate, retrospectively identified Peak Day, P+1-P+3, any permitted interval end, support state, allowed wording, and history eligibility.

**Important** These are synthetic review scenarios, not private user charts. A final acceptance set still requires at least 20 de-identified, licensed or authorized gold charts reviewed against the exact observation fields.

### Core scenarios

| ID   | Scenario                                | Minimum sequence                                                                                             | Reviewer output requested                                                                                        |
|------|-----------------------------------------|--------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------|
| C-01 | Standard dry -> mucus -> Peak -> P+3    | Flow; dry; lower-quality mucus; wet; clear/stretchy/lubricative-type day; three lower observed calendar days | Mark cycle start, pattern boundary, Peak-type days, Peak Day, P+1-P+3, allowed wording, and history eligibility. |
| C-02 | Gap before pattern opening              | Flow; dry; one absent calendar date; first mucus observation                                                 | Does the gap block, qualify, or leave the opening boundary unchanged?                                            |
| C-03 | Gap during three-day sequence           | Peak-type day; one lower observed day; absent date; later lower row                                          | Confirm blocked state, affected outputs, and recovery wording.                                                   |
| C-04 | Explicit not observed                   | Peak-type day; one day intentionally marked not observed; two lower rows                                     | Distinguish intentionally not observed from an absent date, if clinically necessary.                             |
| C-05 | Gradual decline                         | Peak-type day; wet; damp; dry                                                                                | Confirm whether all three days being lower is sufficient and identify the Peak date.                             |
| C-06 | Continuous Peak-type signs              | Clear/stretchy/lubricative-type observations for 10-20 consecutive days                                      | Choose supported, restricted, caution, or no interpretation; provide safe wording.                               |
| C-07 | Continuous non-Peak mucus / BIP-like    | Unchanged sticky/cloudy observations for 30 days                                                             | Define the prerequisite for any interpretation and whether standard summaries must remain suppressed.            |
| C-08 | Non-Peak-only run ending                | One to two lower-quality mucus days then dry; separately three or more then dry                              | Define whether any Peak/plus-count/end concept is permitted or the pattern remains unsupported.                  |
| C-09 | Separated qualifying sequences          | Peak-type; three lower days; later Peak-type; three lower days                                               | Choose first, later, review without selection, or no interpretation.                                             |
| C-10 | High-quality mucus during light flow    | Heavy; moderate; light plus clear/lubricative sign; three lower days                                         | Can the flow day be a candidate, be recorded only, or require review?                                            |
| C-11 | Mid-cycle spotting or brown observation | Isolated spotting/brown observation followed by dry days                                                     | Define recording, boundary, fertility-related meaning, and review wording.                                       |
| C-12 | Cycle-start preamble                    | Light; heavy; moderate                                                                                       | Identify Cycle Day 1 and which days belong in cycle length.                                                      |

| ID   | Scenario                   | Minimum sequence                                                          | Reviewer output requested                                                         |
|------|----------------------------|---------------------------------------------------------------------------|-----------------------------------------------------------------------------------|
| C-13 | Missing date inside flow   | Heavy/moderate Jan 1; no Jan 2 row; heavy/moderate Jan 3                  | Continuing flow, possible new cycle, or uninterpretable?                          |
| C-14 | No heavy/moderate boundary | Complete record containing only light/spotting/brown bleeding             | Create a cycle, leave unbounded, or require review?                               |
| C-15 | Past edit changes marker   | Qualifying sequence; later edit changes a post-candidate day to Peak-type | Confirm recalculation, removal/replacement of the prior marker, and user message. |

## Input-combination scenarios

| ID   | Combination                                 | Reviewer decision requested                                       |
|------|---------------------------------------------|-------------------------------------------------------------------|
| I-01 | Damp with no appearance                     | Mucus, non-mucus sensation, incomplete entry, or other?           |
| I-02 | Wet with no appearance                      | Classification and entry-validity decision.                       |
| I-03 | Shiny with no appearance                    | Classification and entry-validity decision.                       |
| I-04 | Dry plus clear appearance                   | Accept, correct, record without interpretation, or Peak-type?     |
| I-05 | Dry plus lubricative appearance             | Accept, correct, record without interpretation, or Peak-type?     |
| I-06 | Dry plus yellow appearance                  | Valid combination and classification?                             |
| I-07 | Spotting plus clear/lubricative sign        | Primary display and whether it may affect Peak.                   |
| I-08 | Several observations with conflicting signs | Confirm reduction logic and details that must remain visible.     |
| I-09 | Frequency omitted vs. recorded              | Interpretive effect, export-only effect, or required correction?  |
| I-10 | Duplicate, out-of-order, or invalid dates   | Required rejection, recovery, and reviewer-facing representation. |

## Special contexts requiring an explicit scope decision

For each context, select one: ☐ full interpretation ☐ restricted interpretation ☐ defined caution ☐ no interpretation/referral. Then describe prerequisites and permitted wording.

☐ Postpartum and breastfeeding - Decision/limits: \_\_\_\_\_

☐ Perimenopause - Decision/limits: \_\_\_\_\_

☐ Recent hormonal contraception - Decision/limits: \_\_\_\_\_

☐ Persistent discharge or suspected infection - Decision/limits: \_\_\_\_\_

[ ] Irregular or unusual bleeding - Decision/limits: \_\_\_\_\_

[ ] Medications or conditions that may alter signs - Decision/limits:

\_\_\_\_\_

[ ] Very long cycles with several mucus patches - Decision/limits:

\_\_\_\_\_

[ ] Known subfertility - Decision/limits: \_\_\_\_\_

[ ] Fewer than two eligible historical cycles - Decision/limits:

\_\_\_\_\_

### Gold-chart acceptance record

[ ] Reviewer marks each gold chart independently before seeing automated output.

[ ] Product records exact allowed wording and supported population/pattern scope.

[ ] Engineering encodes exact ISO dates and original observation fields, not only synthetic ranks.

[ ] Output is compared field by field; disagreements are adjudicated and logged.

[ ] Approval records reviewer identity/credentials, authorized source edition, date, engine version, and limitations.

### Gold-chart set details

Set identifier / custodian: \_\_\_\_\_

Number of charts reviewed: \_\_\_\_\_

Authorization or license reference:

\_\_\_\_\_

Adjudication record location: \_\_\_\_\_

Exceptions or unresolved charts:

\_\_\_\_\_

## 5. Ordinary-user terminology worksheet

Review the net impression of each term in the context of an unassisted health app. Approval should specify the exact text, required limitation, allowed surface, and supported scope.

| Term                                                    | Interim status      | Reviewer action                                 | Context / concern                                            |
|---------------------------------------------------------|---------------------|-------------------------------------------------|--------------------------------------------------------------|
| Peak-type observation                                   | Current interim use | Approve / revise / prohibit                     | Describes a recorded sign, not ovulation.                    |
| Possible Peak Day                                       | Current interim use | Approve / revise / prohibit                     | Used while the three following days are incomplete.          |
| Well Within marked Cycle Day X as Peak Day              | Current interim use | Approve / revise / prohibit                     | Retrospective software marker after the present rule is met. |
| Your chart shows a post-Peak pattern                    | Current interim use | Approve / revise / prohibit                     | Chart-only statement; no ovulation claim.                    |
| This reflects your chart; it does not confirm ovulation | Current limitation  | Approve / revise / require                      | Shown beside supported retrospective Peak summaries.         |
| A few days need context                                 | Current interim use | Approve / revise / prohibit                     | Missing/absent date limits what the app shows.               |
| More than one possible Peak pattern                     | Current interim use | Approve / revise / prohibit                     | App does not choose between separated qualifying sequences.  |
| Fertile Start / Fertile End / Total fertile days        | Held                | Approve only with defined scope / continue hold | Broad patient-specific boundary terms.                       |
| High / Moderate / Low confidence                        | Removed             | Keep removed / define validated alternative     | No calibrated probability supports these labels.             |
| Most fertile day confirmed / past the fertile window    | Held/removed        | Continue hold / provide approved alternative    | Can imply biological certainty or decision guidance.         |

**Non-negotiable interim boundary** Until separately approved, Well Within will not present ovulation confirmation, pregnancy probability, diagnosis, treatment, 'safe'/'infertile' claims, or avoiding-pregnancy instructions.

### Approved wording record

Observation-level term: \_\_\_\_\_

Developing-pattern term: \_\_\_\_\_

Retrospective Peak Day statement:

\_\_\_\_\_

Required chart-only / ovulation limitation:

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Missing-data statement: \_\_\_\_\_

Multiple-sequence statement:

---

Unsupported-context statement:

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Terms prohibited for ordinary users:

---

## 6. Source and evidence register

Please replace broad web support with the exact authorized source edition and page whenever approving a method-specific rule. The sources below provide context; none independently validates Well Within.

1. [Creighton Model - About](#) - Standardized, practitioner-taught system and individualized follow-up; does not validate this app.
2. [Creighton Model - Background](#) - Public overview of mucus progression, Peak, long-cycle patches, and BIP complexity; not a software license.
3. [Saint Paul VI Institute public chart/IP sample](#) - Public sample of selected concepts plus protected-material warnings; not a complete specification or license.
4. [FertilityCare Centers of America - Creighton Model System](#) - Current description of a standardized, practitioner-taught system; no approval or affiliation with Well Within.
5. [Peak Day selected by woman, expert, and computer](#) - Shows that Peak identification can differ; does not validate Well Within's algorithm.
6. [Cervical mucus patterns and the fertile window](#) - Trained-participant observations and non-Peak-only pattern nuance; not universal app validation.
7. [Peak Day validation against a hormone monitor](#) - Trained-instruction context and limits; does not approve Well Within wording or classifications.
8. [CDC U.S. MEC - Fertility Awareness-Based Methods](#) - Identifies contexts requiring delay or special counseling; does not define this app's method rules.

### Internal implementation evidence supplied with this packet

|                       |                                                                                                              |
|-----------------------|--------------------------------------------------------------------------------------------------------------|
| Rule inventory        | RULE_PROVENANCE_MATRIX.md                                                                                    |
| Claim inventory       | CLAIM_REVIEW_MATRIX.md                                                                                       |
| Fixture inventory     | CLINICAL_REVIEW_FIXTURES.md                                                                                  |
| Current specification | RULES_ENGINE_SPEC.md and CURRENT_CYCLE_SUMMARY_MATRIX.md                                                     |
| Implementation record | Phase 1A and Phase 1B implementation notes; commit 261fc0f                                                   |
| Automated evidence    | Core/mobile test suites, type checks, lint/build, production bundle, simulator review, and passing GitHub CI |

**Separate counsel track** Trademark, copyright/license, intended-use, FDA, FTC, privacy, security, App Store metadata, and public-claim substantiation are not within this clinical sign-off. Flag concerns, but do not treat this form as legal approval.

## 7. Reviewer attestation and next-step record

Sign-off applies only to the decisions and limitations explicitly recorded in this packet. Blank items remain unapproved.

|                                 |  |
|---------------------------------|--|
| Reviewer name                   |  |
| Credentials / certification     |  |
| Current role / organization     |  |
| Relevant clinical experience    |  |
| Authorized source edition(s)    |  |
| Conflict-of-interest disclosure |  |
| Review date                     |  |

### Attestation

I reviewed the current software behavior, priority decision records, synthetic fixtures, terminology worksheet, and stated limitations. My decisions are limited to the scope and sources I documented. I understand that this review does not grant trademark/copyright rights, establish regulatory status, approve privacy/security claims, or create affiliation with a method owner or training organization.

Reviewer signature: \_\_\_\_\_

Date: \_\_\_\_\_

Product owner acknowledgement:

\_\_\_\_\_

Engineering decision-record version:

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### Required follow-up after review

[ ] Product converts each reviewer decision into a versioned rule/claim record with exact scope and wording.

[ ] Counsel separately reviews method naming, source/license provenance, intended use, public claims, privacy, and platform metadata.

[ ] Engineering updates source, specification, fixtures, tests, presentation model, and user copy together - never one in isolation.

[ ] The reviewer rechecks changed gold fixtures and rendered user-facing copy before release.

[ ] The release record preserves reviewer, source edition, app/engine version, limitations, and re-review trigger.

**Completion rule** No blank, held, or disputed clinical decision may be treated as approved merely because the current automated tests pass.